

Patient ID: RSanchezPeriod: September 17 to October 17, 2024Report Date: October 18, 2024Coverage: Bed: 605/730h (82.9%)
Chair: 522/730h (71.5%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm
Very High HR > 110 bpm

Low HR < 45 bpm
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm
High Breathing > 30 brpm

High HR > 100 bpm
Very High Breathing > 40 brpm

Last 24 Hours

Elevated Breathing 9 AM to 10 AM			
Vital Stat	Avg	Min	Max
Heart Rate	57 bpm	50 bpm	85 bpm
Breathing	16 brpm	10 brpm	46 brpm

Last 24 hours: one elevated breathing episode in the morning, otherwise within normal range.

Episodic Events (last 24h)

Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
9 AM to 10 AM	1h	1	Elevated Breathing	28	13	46	Check SpO2 and lung sounds; assess for fluid overload or early infection

Patient clinical status over 31 days from Sep 17 to Oct 17

HR

Breathing

Elevated HR Oct 05 to Oct 07 (3d)	Elevated Breathing Sep 25 to Sep 29 (5d)	Elevated Breathing Oct 06 to Oct 11 (6d)	High HR Oct 05 to Oct 07 (3d)	High Breathing Sep 24 to Oct 03 (10d)	High Breathing Oct 08 to Oct 09 (2d)
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43 episodic events Detected, spanning 50h total. Conditions: Elevated Breathing, Elevated Heart Rate, High Breathing, High Heart Rate, Very High Breathing. Priority grade: Medium

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
43	50h	High Breathing	1	100%

Major Findings: Sustained high breathing (avg 35, peak 58)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Sep 24	1 PM to 2 PM	15h	12	High Breathing	35	12	58	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Oct 05	2 AM to 3 AM	3h	2	High Heart Rate	104	87	115	Check temp, hydration, pain; review for arrhythmia or infection
Oct 08	8 AM to 9 AM	3h	3	High Breathing	35	12	54	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Sep 25	7 AM to 8 AM	6h	6	Elevated Breathing	26	14	57	Check SpO2 and lung sounds; assess for fluid overload or early infection
Oct 05	1 AM to 2 AM	2h	2	Elevated Heart Rate	98	57	108	Check temp, hydration, pain; review for arrhythmia or infection
Oct 06	1 PM to 2 PM	9h	8	Elevated Breathing	26	13	46	Check SpO2 and lung sounds; assess for fluid overload or early infection
Oct 08	7 AM to 8 AM	2h	2	Very High Breathing (brief)	46	14	54	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

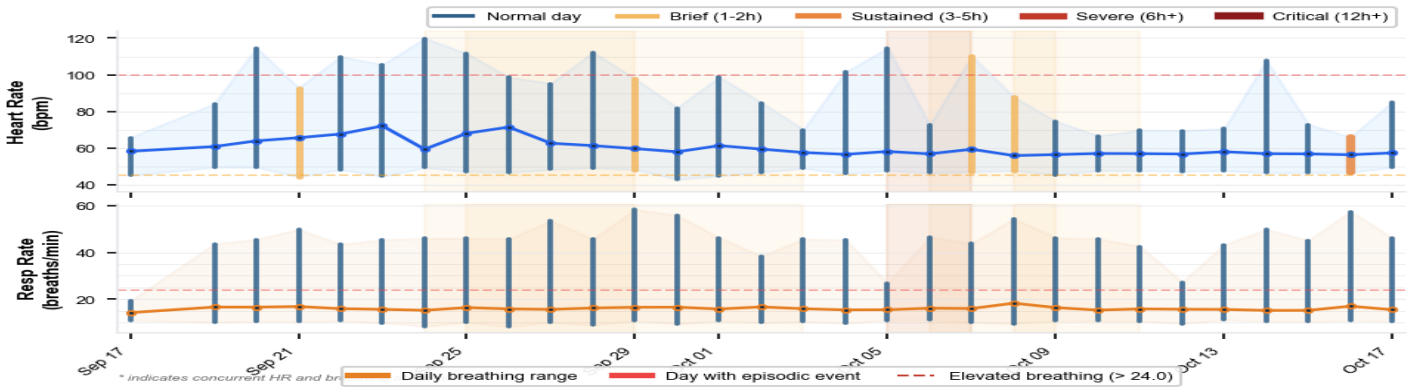
The P5 to P95 heart rate spread was 33 bpm (52 to 85), indicating significant cardiac variability.

Suggested Clinical Review Actions

- High Breathing (avg 35 brpm, peak 58 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Elevated Breathing (avg 26 brpm, peak 46 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Heart Rate (avg 104 bpm, peak 115 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Elevated Heart Rate (avg 98 bpm, peak 108 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Very High Breathing (avg 46 brpm, peak 54 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

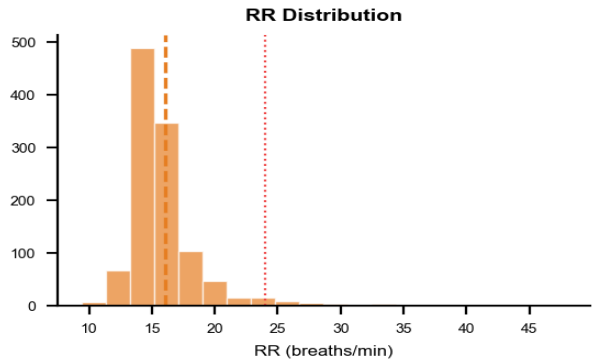
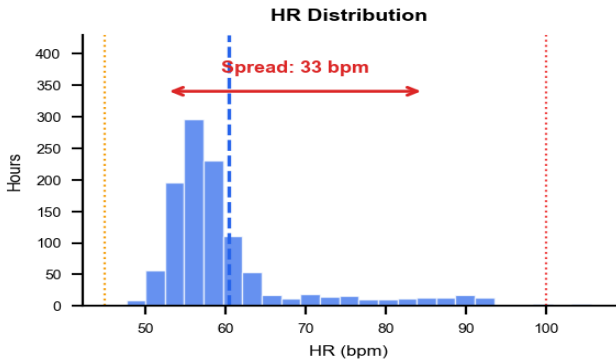
Clinical Pattern Observations:

- 1. **High variability:** HR spread 32 bpm (52 to 85).
- 2. **Clustered pattern:** Episodic events on 5 of 31 days (16%).
- 3. **Daytime pattern:** Episodes concentrated during waking hours.

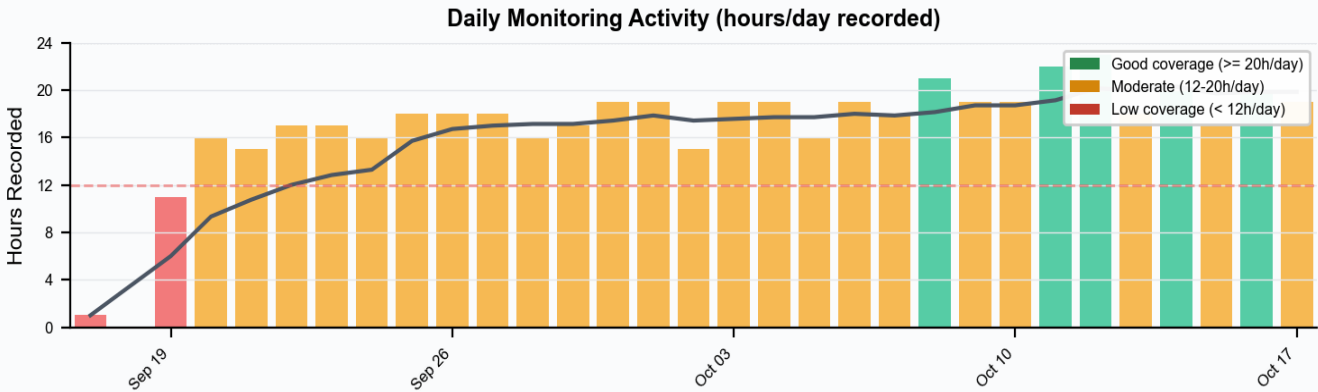


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.